

ETHIOPIAN HEALTH PROFESSIONALS LICENSING EXAMINATION(EHPLE) PRACTICE TEST BOOKLET 2025

PEDIATRIC & CHILD HEALTH NURSING



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MINISTRY OF HEALTH-ETHIOPIA

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HEALTHIER CITIZENS FOR PROSPEROUS NATION!



Message from the State Minister, Ministry of Health – Ethiopia



Mrs. Frehiwot Abebe

Improving the quality of healthcare remains a critical global priority, essential for achieving sustainable development and universal health coverage. One of the most effective ways to ensure high standards in healthcare delivery is through the consistent assessment of professional competencies. In line with this commitment, the Ministry of Health of Ethiopia launched the Ethiopian Health Professionals Licensing Examination (EHPLE) in 2019 for undergraduates in seven core health disciplines. Since its inception, the program has expanded to encompass 17 health professions, reflecting our continued effort to build a competent and responsive health workforce.

The EHPLE was established with the primary aim of identifying and certifying health professionals who possess the minimum competencies required to provide safe and effective care. This ensures that all practicing professionals are equipped to meet the expectations of the communities they serve. The exam is coordinated by the Health and Health-Related Institutions and Professionals' Regulatory Lead Executive Office (LEO), which consists of four specialized desks. The LEO plays a vital role in system strengthening, enabling the exam to be conducted with increased efficiency, organization, and quality assurance.

It is important to recognize that the EHPLE is not a conventional academic or recruitment examination. Rather, it is a rigorous, competency-based assessment designed to evaluate the practical skills and critical thinking abilities that are essential for professional practice. This *Practice Items Booklet* is part of our broader strategy to support examinees and educators in preparing for the licensing process. It offers a realistic overview of the exam structure, promotes a deeper understanding of its objectives, and fosters transparency and sustainability in implementation.

The development of this booklet has been made possible through the collaborative efforts of experts from higher education institutions, the Addis Ababa University Testing Center, and professional associations. Their insights and dedication have been instrumental in shaping this resource. I extend my heartfelt thanks to all contributors and the Ministry of Health team for their tireless work and steadfast commitment to excellence in healthcare regulation.

We believe this booklet will serve as a valuable tool for candidates preparing for the examination and for educators guiding them toward success. Together, we are building a stronger, more competent health workforce for Ethiopia.

Introduction EHPLE

Welcome to the 2025 Edition of the Ethiopian Health Professionals Licensing Examination Practice Test Booklet.

This resource is designed to support candidates in their preparation for the national licensing examination, ensuring they are well-equipped with the knowledge and clinical reasoning skills required to deliver competent and ethical healthcare across Ethiopia.

In Ethiopia, the licensing examination serves as a pivotal tool in evaluating the competency of health professionals who have completed their academic training and are ready to transition into independent practice. The exam not only ensures public safety by verifying professional readiness but also upholds national and international standards in healthcare service delivery. As the healthcare system continues to expand and evolve, a strong, well-regulated professional workforce becomes even more essential.

This practice booklet contains 200 multiple-choice questions modelled after the structure, content, and format of the actual licensing examination. The questions span across core clinical domains essential for entry-level practice. This includes areas such as Patient care, leadership and management, ethics, and Health Promotion, as outlined in the 2024 Information Booklet.

The booklet is intended to:

- Familiarize candidates with the structure and cognitive level of the actual licensing examination.
- Offer opportunities for self-assessment and identification of strengths and gaps in knowledge.
- Encourage critical thinking and clinical decision-making in scenarios reflective of real-world healthcare practice in Ethiopia.
- Reduce exam-related anxiety by providing realistic practice experiences.

Administered under the auspices of the Ministry of Health and relevant regulatory bodies, the Ethiopian Health Professionals Licensing Examination is grounded in evidence-based frameworks and aligned with both national needs and global best practices. It has been structured to assess a range of competencies, including medical knowledge, communication, professionalism, and safe clinical practice.

This practice test is not a predictive tool, and performance on it does not guarantee similar outcomes on the actual exam. However, it provides an invaluable opportunity for familiarization with question styles, pacing strategies, and content review. We strongly recommend that candidates simulate real testing conditions while using this resource, including timing their responses and reviewing rationale for both correct and incorrect answers.

The goal of licensing examinations is not only to regulate entry into the profession but also to inspire excellence and lifelong learning. By engaging with this practice test, you are taking a proactive step toward professional development, accountability, and a commitment to serving your patients with competence and compassion.

We wish you success on your licensing journey and beyond.

Acknowledgements

This *Practice Test Items Booklet for the Ethiopian Health Professionals Licensing Examination* is the result of a collaborative effort among dedicated professionals committed to advancing the quality of health sciences education in Ethiopia. The Ministry of Health extends its sincere appreciation to all those who contributed their time, expertise, and insights to the development of this essential resource.

Special recognition is given to the item developers and reviewers from various higher education institutions, both public and private, whose academic rigor and practical experience were instrumental in shaping the content and ensuring its relevance to real-world healthcare practice. Their contributions have helped create a high-quality, standardized practice tool that reflects national competencies and global benchmarks.

We are especially grateful to the team at the Addis Ababa University Testing Center, whose technical support and academic guidance were critical throughout the process. Their leadership in assessment design and educational evaluation has significantly strengthened the quality of this booklet.

We also thank the committed staff of the Ministry of Health, particularly those within the Health and Health-Related Institutions and Professionals' Regulatory Lead Executive Office (HHRIPR LEOs), whose coordination, oversight, and unwavering dedication ensured the successful completion of this project.

Together, these collective efforts reflect our shared vision of building a competent, ethical, and resilient health workforce for Ethiopia.

Pediatric And Child Health Nursing

1. A six-year-old school-age male child was brought to a pediatric clinic by his mother with complaints of cough and nasal congestion for a three-day duration. On assessment, he had a history of contact with his school friend who developed a common cold. Otherwise, there is no history of vomiting and breathing difficulty.
What additional assessment findings would support the diagnoses?
A. Watery nasal discharge
B. Wheezing and stridulous sound
C. Nasal flaring and subcostal retraction
D. Barking cough and hoarseness of voice
ANSWER: A
2. A four-month-old abandoned female child presented to a pediatric clinic with a runny nose and cough lasting for one day. Upon examination, she exhibits wheezing throughout her lung fields, accompanied by fine crackles. Otherwise, all vital signs are stable.
What is the most likely etiology of the above case?
A. Pneumococcus
B. Para influenza virus
C. Respiratory syncytial virus
D. Haemophilus influenzae type b
ANSWER: C
3. A six-year-old boy was admitted to a pediatric department after presenting with cough, fever, and breathing difficulty for two days. His body temperature is 37.7 degrees Fahrenheit; respiratory rate is 32, and oxygen saturation is 87 percent on room air. On physical examination, he has crackles and wheezing upon auscultation.
What is the priority nursing diagnosis for the above scenario?
A. Risk of aspiration due to unswallowed saliva
B. Activity Intolerance related to difficulty of breath
C. Anxiety related to a threat to or change in health status
D. Ineffective Airway Clearance related to bronchial secretions
ANSWER: D
4. A four-year-old male child was brought to a pediatric emergency department with complaints of a productive cough and fever of two days duration. His mother also reported that the child has chills and shortness of breath. His vital signs: temperature is 38.7degree centigrade, pulse rate is 144 beats/ min, and respiratory rate is 47 breaths /min. On assessment, the nurse noted crackles.
What is the most likely nursing intervention in this case?
A. Monitor serial chest x-rays
B. Provide intravenous fluids
C. Monitor serial arterial blood gases
D. Administer prescribed medications
ANSWER: D

5. A 13-year-old boy was brought to a pediatric clinic by his father with complaints of shortness of breath and cough for a three-day duration. On assessment, the nurse noted wheezes throughout the lung fields and sub-costal retractions. Vital signs: temperature is 36.4 degrees centigrade; pulse rate is 110/min; respirations rate is 37/min, and 95 percent oxygen saturation.
What is the priority nursing management for the above scenario?
- A. Give daily maintenance fluid
 - B. Administer a salbutamol puff
 - C. Administer intranasal oxygen
 - D. Assess skin color every hour
- ANSWER: B
6. A four-month-old female child was presented to a pediatric clinic with several days of nasal congestion, rhinorrhea, and fever. On physical examination, the nurse noted mildly elevated body temperature, diffuse wheezing, and subcostal retractions.
What is the most likely nursing management in this case?
- A. Give inhaled epinephrine
 - B. Do not give the child anything by mouth
 - C. Give antipyretics and antibiotics
 - D. Adequate hydration and nasal suctioning
- ANSWER: D
7. A five-year-old female child was admitted to a pediatric ward after she presented with a cough of five days' duration. Her respiratory rate is 64 breaths/min, and her temperature is 38.6 degrees centigrade. On physical examination, a nurse noticed that the child had nasal flaring, lower chest wall indrawing, and crackles. A physician ordered the nurse to deliver oxygen by a simple face mask.
What is the preferred flow rate that the nurse should adjust the oxygen concentration to?
- A. 2-5 L/min
 - B. 6-10 L/min
 - C. 11-15 L/min
 - D. 16-20 L/min
- ANSWER: B
8. A ward nurse is evaluating the prognosis of a four-year-old female child who has been on treatment for severe community-acquired pneumonia for three days. The nurse noted a dull chest on both sides.
What is the most likely complication the child had?
- A. Emphysema
 - B. Lung abscess
 - C. Pleural effusion
 - D. Pneumothorax
- ANSWER: C

9. A seven-year-old girl visited a pediatric emergency department with a complaint of a cough for a day. On physical examination, a nurse noticed that her respiratory rate is 54 breaths/ min and oxygen saturation is 82 percent at room air. Then, the nurse administered oxygen with a flow rate of 2 liters/min with nasal prongs.
What is the most likely condition that implies the improvement of the child?
A. Increase in the work of breathing
B. Increase in oxygen saturation
C. Increase in respiratory rate
D. Increase in pulse rate
ANSWER: B
10. A three-year-old female child is presented to a pediatric clinic with a barking cough of two days. On assessment, her body temperature is mildly elevated, and chest examination reveals inspiratory stridor.
What is the most likely medical diagnosis of the child?
A. Pneumonia
B. Epiglottitis
C. Pharyngitis
D. Croup syndrome
ANSWER: D
11. A four-year-old female child was brought to a pediatric emergency department with joint pain for a three-day duration. She had a history of tonsillitis two weeks ago. Currently, she has free mobile non-tender subcutaneous nodules over the wrist and elbow joints.
What additional evidence would support fulfilling the diagnostic criteria?
A. Fever
B. Elevated ESR
C. Sydenham chorea
D. PR prolongation on ECG
ANSWER: C
12. A six-year-old female child was brought to a pediatric clinic by his mother with chief complaints of easy fatigability and shortness of breath for a week's duration. On examination, she has severe palmar pallor and pale conjunctivae.
What is the most likely expected laboratory finding in the above scenario?
A. Increased WBC count
B. Elevated cardiac markers
C. Elevated serum creatinine
D. Decrease in hemoglobin level
ANSWER: D
13. A nurse is taking care of a seven-year-old male child who has been on monthly benzathine penicillin injections for carditis secondary to acute rheumatic fever. The child's mother asked the nurse for the duration of the treatment.
What is the most appropriate response of the nurse?
A. Until age 21 years or a minimum of 5 years
B. Until age 21 years or a minimum of 10 years
C. Until age 40 years or a minimum of 5 years
D. Until age 40 years or a minimum of 10 years
ANSWER: B

14. A 10-year-old male child is presented to a pediatric clinic with easy fatigability and shortness of breath for a week's duration. On assessment, the child's conjunctivae and palms appear pale. Laboratory result reveals: hemoglobin 10 g/dl, Mean Corpuscular Volume of 65 fl (normal range 77–95 fl), and occult blood in stool.
What is the most appropriate nursing management in this case?
- A. Give folic acid as ordered
 - B. Give intravenous fluid as ordered
 - C. Give iron supplements as ordered
 - D. Transfuse with packed RBC as ordered
- ANSWER: C
15. A ten-year-old male child was admitted to a pediatric emergency department with the diagnosis of heart failure. A ward physician ordered furosemide 2 mg/kg IV in two divided doses. A nurse in charge prepares nursing care to follow him on treatment.
What is the most appropriate nursing management in this case?
- A. Record input and output
 - B. Encourage increased-calorie intake
 - C. Encourage foods low in potassium
 - D. Observe for side effects like vomiting and diarrhea
- ANSWER: A
16. A one-year-old male child accompanied by his mother comes to the hospital with complaints of shortness of breath and bluish discoloration of the lips and tongue. Laboratory result reveals a Hematocrit level above 99 percent. The Echocardiogram confirmed tetralogy of Fallot.
What most likely complication the child experiences?
- A. Polycythemia
 - B. Pulmonary edema
 - C. Congestive heart failure
 - D. Cyanotic congenital heart disease
- ANSWER: A
17. A 16-year-old strict vegetarian female adolescent came to the hospital with complaints of loss of appetite and diarrhea for a week's duration. On assessment, she has paresthesia, pale conjunctivae, hyporeflexia, beefy tongue, and a positive Babinski reflex test.
What is the most likely medical diagnosis of the child?
- A. Sickle cell anemia
 - B. Iron deficiency anemia
 - C. Folic Acid deficiency anemia
 - D. Vitamin B12 deficiency anemia
- ANSWER: D
18. A pediatric nurse is caring for a two-month-old infant who has been admitted with a diagnosis of pyloric stenosis. The nurse has been providing and monitoring intravenous infusion and fluid replacement therapy preoperatively.
What is the initial expected treatment outcome for this infant condition?
- A. The baby starts to breastfeed
 - B. Bulged fontanel
 - C. Moist lips
 - D. Stable urine output
- ANSWER: C

19. A four-year-old child was brought to an under-five OPD with complaints of diarrhea and vomiting of a day's duration. On assessment, the child had a history of watery and frequent diarrhea, sunken eyeballs, and irritability following diarrhea and vomiting, severe wasting, and poor skin turgor. What is the most likely sign or symptom that could help to classify dehydration in this case?
- A. Poor skin turgor was noted by the nurse
 - B. Severe wasting was noted by the mother
 - C. A stable vital sign by the nurse
 - D. Sunken eye. The ball noted by the mother
- ANSWER: D
20. A seven-year-old female child was presented to a referral hospital with a complaint of a mass protrusion in her labia majora for one month. On assessment, the child had a feeling of intolerance, temperature of 37.2 degrees centigrade, respiratory rate of 25 breaths/ min, and abdominal distention. A pediatric nurse suspected the child was suffering from an incarcerated inguinal hernia. What is the most likely additional symptom that supports the diagnosis of this child?
- A. Bloody stool
 - B. Tender mass
 - C. Bilious vomiting
 - D. Non-reducible mass
- ANSWER: D
21. A nurse is caring for a two-year-old child who has been admitted to an emergency ward with a diagnosis of intussusception. A pediatrician has ordered to administer a therapeutic enema. However, a pediatric nurse noted that the child has a weak pulse, high temperature, low blood pressure, and rebound tenderness. Then, the nurse withheld the order and notified the physician. What is the most likely justification for the nurse to withhold the order?
- A. Operative reduction is better
 - B. The child is in shock state
 - C. A therapeutic enema is not appropriate
 - D. The obstruction is resolved
- ANSWER: B
22. A 12-year-old child was brought to a pediatric emergency department with complaints of nausea, vomiting, and severe abdominal pain of one day duration. A pediatric surgeon diagnosed the child with acute appendicitis and ordered to prepare for emergency surgery. What is the priority preoperative nursing action for this child?
- A. Follow the urine output
 - B. Urinary catheterization
 - C. Provide anti-pain
 - D. Fluid rehydration
- ANSWER: B
23. A pediatric nurse is caring for a five-year-old child who has been diagnosed with intussusception and is planned for hydrostatic reduction as an effective management option for the child. What is the initial nursing intervention for the preparation of hydrostatic reduction?
- A. Assess for worsening abdominal pain
 - B. Placing the infant on NPO status
 - C. Inserting a nasogastric tube
 - D. Monitor the infant's vital signs
- ANSWER: B

24. A pediatric nurse is caring for an eight-month-old infant who has been diagnosed with severe dehydration in the emergency ward. The pediatrician ordered to resuscitate the infant according to the option C protocol, and the nurse did the same. The infant's condition improved. What is the appropriate nursing reassessment time to evaluate the outcome?
- A. One hour
 - B. Three hours
 - C. Six hours
 - D. Four-hours
- ANSWER: C
25. A four-year-old child was brought to a hospital with complaints of profuse watery diarrhea and vomiting of two days' duration. On assessment, the child was unable to take food and fluids despite the frequent losses. A nurse diagnosed the child with a severe complication. What is the most likely complication that occurred in this child?
- A. Peritonitis
 - B. Dehydration
 - C. Infections
 - D. Recurrent diarrhea
- ANSWER: B
26. A pediatric nurse is assessing a three-year-old child who presented to a pediatric emergency department with a complaint of sudden, severe paroxysmal colicky pain. On assessment, the child had a history of passage of currant jelly stool and a palpable, tender mass at the right upper abdomen. What is the most likely medical diagnosis of this case?
- A. Constipation
 - B. Appendicitis
 - C. Intussusceptions
 - D. Enterocolitis
- ANSWER: C
27. A pediatric nurse working at an under-five OPD in a health center diagnosed a four-year-old child with Giardiasis and planned to provide a safe and effective drug. What is the first-line drug of choice for this case?
- A. Albendazole
 - B. Tinidazole
 - C. Mebendazole
 - D. Paromomycin
- ANSWER: B
28. A five-year-old child has been admitted to a pediatric surgical ward with the diagnosis of a strangulated inguinal hernia. A surgical team discussed the appropriate management for the child's condition. What is the definitive management for the child?
- A. Administration of broad-spectrum antibiotics
 - B. Administration of intravenous fluids
 - C. Reduction of the abdominal content
 - D. Surgical removal of the protruded part
- ANSWER: D

29. A pediatric nurse is caring for a five-year-old child who is admitted to a pediatric ward with the diagnosis of acute gastroenteritis and severe dehydration of two days duration. On assessment, the anthropometric measurements revealed that the child was neither wasted nor stunted. What is the appropriate fluid for the management of this child's case?
- A. Ringer lactate solution
 - B. 5%Dextrose solution
 - C. Oral rehydration salt
 - D. ReSomal solution
- ANSWER: A
30. A five-year-old child was brought to a health center with complaints of vomiting and diarrhea of one day duration. A nurse in charge has assessed and diagnosed the child with some dehydration. The nurse put the child on resuscitation as per plan B dehydration treatment protocol. What is the most likely expected outcome after 4 hours for this child's case?
- A. Decreased diarrheal episodes
 - B. Decreased vomiting
 - C. No sign of dehydration
 - D. Sign of some dehydration
- ANSWER: C
31. A 10-year-old girl who was sexually assaulted two months ago is brought to a hospital to seek medical advice. On assessment, a nurse noted that she often engages in inappropriate coping behaviors. What is the most likely manifestation that suggests sexual abuse in this girl?
- A. Feeling powerful
 - B. Feeling loved
 - C. Having high self-esteem
 - D. Self-mutilation
- ANSWER: D
32. A nurse is caring for a 17-year-old adolescent girl diagnosed with vaginal discharge syndrome secondary. On assessment, she is an orphan and works as a commercial sex worker. The nurse provided treatment based on the protocol. What is the most important nursing management for this adolescent?
- A. Contact tracing and management
 - B. Link for financial support
 - C. Promotion of condom use
 - D. Providing legal support
- ANSWER: C
33. A 15-year-old male adolescent who has been diagnosed with urethral discharge syndrome secondary to Urethritis. The adolescent had received treatment a week ago and presented to the hospital for a follow-up visit. A nurse assessed the patient's response to the treatment. What is the most likely condition that suggests the adolescent has not shown any improvement?
- A. Burning sensation during micturition
 - B. Testicular pain and swelling
 - C. Polyarthralgia
 - D. Genital ulcer
- ANSWER: A

34. An 11-year-old adolescent girl was admitted to a hospital with a diagnosis of gonococcal infection. On assessment, the adolescent girl appears sad and withdrawn and has a history of abuse. What is the most likely abuse that the adolescent girl had experienced?
- A. Neglect
 - B. Physical
 - C. Psychological
 - D. Sexual
- ANSWER: D
35. A three-year-old child was brought to a pediatric clinic by his mother with complaints of fever and difficulty swallowing for a day's duration. Upon physical examination, a pediatric nurse noticed that his tonsils are red and swollen bilaterally and touch each other just below the uvula. What is the most likely degree of tonsil enlargement in this case?
- A. Grade one
 - B. Grade two
 - C. Grade three
 - D. Grade four
- ANSWER: D
36. A 10-year-old female child was brought to a pediatric clinic with complaints of earache and right ear discharge for a week's duration. On the Weber test, the child hears sound better in the right ear than in the left ear. What is the most likely interpretation of the above examination?
- A. Conductive hearing loss in the right ear
 - B. Conductive hearing loss in the left ear
 - C. Sensory-neural hearing loss in both ears
 - D. Sensory-neural hearing loss in the right ear
- ANSWER: A
37. A four-year-old male child visited a pediatric clinic with complaints of fever and difficulty swallowing for a three-day duration. Upon physical examination, a pediatric nurse noticed symmetrically enlarged and red tonsils. What is the first line of nursing management for the above case?
- A. Prepare for surgery
 - B. Give Analgesics as ordered
 - C. Give antibiotics as ordered
 - D. Give Corticosteroid as ordered
- ANSWER: C
38. A seven-year-old male child visits a pediatric clinic with complaints of fever, ear pain, and discharge for a week. In addition, he had a history of cough and difficulty swallowing for the last two weeks. Upon examination, he is lethargic, and visual inspection of the tympanic membrane revealed bulged and purulent erythematic effusion. What is the most likely nursing management for this problem?
- A. Give analgesics as ordered
 - B. Give Antibiotics as ordered
 - C. Prepare for myringotomy
 - D. Give Steroids as ordered
- ANSWER: B

39. A 12-year-old male adolescent came to a pediatric clinic with complaints of sore throat, fever, and difficulty swallowing for a seven-day duration. He had a recent history of acute tonsillopharyngitis. On physical examination, his body temperature is 39 degrees centigrade, and an asymmetric bulge lateral to the uvula with displacement of the uvula.
What is the most likely complication that the child developed?
A. Stomatitis
B. Adenoiditis
C. Peritonsillar abscess
D. Chronic Pharyngitis
ANSWER: C
40. A five-year-old male child was brought by his mother to an under-five OPD with complaints of bilateral leg swelling, decreased urine output, and flank pain for a day's duration. In history, his mother reported that he had bloody and painful urination.
What is the most likely risk factor in the above scenario?
A. History of liver problems
B. History of tonsillopharyngitis
C. Family history of renal problems
D. History of congestive heart failure
ANSWER: B
41. A nine-year-old female child was brought to an under-five OPD by her mother with complaints of swelling of her legs, hands, and face for a three-day duration. Upon physical examination, a pediatric nurse revealed an indentation of 10mm while gently pressing her swollen lower legs.
What is the most appropriate interpretation of the child's physical examination findings?
A. Grade two non-pitting edema
B. Grade two pitting edema
C. Grade three non-pitting edema
D. Grade three pitting edema
ANSWER: B
42. An eight-year-old male child was brought to an under-five OPD by his mother. She reported that the child's eyes have been swollen and red since yesterday. His hands and feet seem to be "puffy" as the day goes on. His appetite has been poor. He appears pale and lethargic.
What is the priority nursing diagnosis of this child?
A. Imbalanced nutrition
B. Impaired skin integrity
C. Ineffective breathing pattern
D. Fluid and electrolyte imbalance
ANSWER: D

43. A four-year-old male child was brought to the pediatrics emergency department by his mother following a burn injury. On assessment, he has decreased urine output, and his eyes are sunken. Upon physical examination, a pediatric nurse noticed palmar pallor, and the child is lethargic. Besides, he has periorbital edema, and skin pinch goes back very slowly. Laboratory examination revealed a sodium level of 140 mEq/L, a potassium level of 5.7 mEq/L, and a creatinine level of 1.5.

What is the most likely initial management strategy for this child?

- A. Administration of diuretics
- B. Administration of antibiotics
- C. Administration of fluid and electrolytes
- D. Administration of sodium bicarbonate

ANSWER: C

44. A seven-year-old male child was brought to the pediatrics clinic by his father with complaints of hematuria and diminished urinary output for a day's duration. On assessment, he has facial edema and is easily fatigable. Her blood pressure is elevated. Laboratory investigation revealed proteinuria (++) and increased serum creatinine level.

What is the most likely nursing management for the above case?

- A. Increase fluid consumption
- B. Reduce high-protein diets
- C. Increase carbohydrate intake
- D. Involved in extraneous physical activities

ANSWER: B

45. A six-year-old male child is admitted to a pediatric ward after presenting with complaints of decreased urine output and generalized body swelling. In history, the swelling started from the periorbital area. Laboratory investigation revealed decreased glomerular filtration rate, an increase in serum creatinine, and blood urea nitrogen. In addition, the electrocardiographic report showed a prolonged QRS complex, a depressed ST segment, high-peaked T waves, and bradycardia.

What is the most likely electrolyte disturbance in the above scenario?

- A. Hyperkalemia
- B. Hyponatremia
- C. Hypercalcemia
- D. Hyperphosphatemia

ANSWER: A

46. A 13-year-old male adolescent is brought to a hospital by his father two weeks after renal transplantation with complaints of fever, swelling and graft pain, and decreased urine output. Upon assessment, he has weight gain, elevated blood pressure, increased blood urea nitrogen, and serum creatinine level.

What is the most appropriate initial management for this child?

- A. Peritoneal dialysis
- B. Broad-spectrum antibiotics
- C. Antihypertensive medications
- D. Immunosuppressant medications

ANSWER: D

47. A nine-year-old female child visited a pediatric clinic with complaints of abdominal cramps, weight loss, and headache. From history, she had renal disease and has been taking diuretics. She has a history of muscle weakness, abdominal cramps, and vomiting. Upon physical examination, she is lethargic, her skin is dry, and she has poor skin turgor.
What is the most likely electrolyte disturbance the child has developed?
A. Hypokalemia
B. Hyponatremia
C. Hypocalcemia
D. Hypophosphatemia
ANSWER: B
48. A three-year-old male child is brought to a pediatric clinic with complaints of difficulty walking and has failed to speak recognizable words since birth. On assessment, he has floppiness in his right hand and leg. His hand is more dysfunctional than his leg. He also has sensory deficits in his right extremities.
What is the most likely type of disability the child developed?
A. Diplegia
B. Para plegia
C. Quadriplegia
D. Hemiplegia
ANSWER: D
49. A four-year-old female child was brought to a pediatric clinic by her father with a complaint of involuntary jerking hand movement. On examination, she is unable to perform fine coordinated motions, the finger-to-nose test, and rapid alternating movements. The child drools saliva, and her speech is difficult to understand.
What is the priority nursing intervention in the above case?
A. Reassuring the family
B. Curing the child's problem
C. Promoting independence and self-care
D. Preventing the progression of the problem
ANSWER: C
50. A five-year-old male child was brought to a pediatric clinic by his father with complaints of involuntary body movement and walking difficulty. On assessment, he has floppiness in his left hand and leg. His mother stated that the child has a language delay compared to her older son. He feels stiffness while dressing. On examination, he has a positive Babinski reflex, ankle clonus, and exaggerated stretch reflexes.
What is the most likely expected outcome after management?
A. Correct associated defects effectively
B. Attain the child's potential within the limits of the problem
C. Gain optimal appearance and integration of motor functions
D. Get educational achievement similar to his healthy friends
ANSWER: B

51. A two-year-old female child was brought to a pediatrics clinic by his mother with complaints of cough and fever for a seven-day duration. On examination, she has a rounded and small skull, small and low-set ears, a depressed nasal bridge, and a single transverse palmar crease. What is the most likely medical diagnosis in this case?
- A. Cerebral palsy
 - B. Down syndrome
 - C. Turner syndrome
 - D. Autism spectrum syndrome
- ANSWER: B
52. A 6-year-old child has been presented to a health center with complaints of cough and low-grade fever for three weeks. On assessment, the child has a history of weight loss, travel to nearby villages, contact with a recently diagnosed person with tuberculosis, consumption of cow milk, unknown vaccination status, and lives in a crowded household with many family members sharing a small living space. What is the most likely risk factor that contributed to the child's condition?
- A. Consumption of cow milk
 - B. Exposure to an infected person
 - C. Recent travel to high TB-burden areas
 - D. unknown vaccination status
- ANSWER: B
53. A 6-year-old child was brought to a pediatric clinic with a complaint of failure to gain weight of two months duration. On assessment, the mother disclosed that the child had an HIV-positive status, recurrent respiratory infections, and had been failing to thrive. What is the priority nursing intervention for the child's condition?
- A. Give prophylaxis to prevent new infections
 - B. Link to nutritional therapy
 - C. Promote comfort
 - D. Provide information about the disease
- ANSWER: B
54. A pediatric nurse is monitoring a 9-year-old child who has been taking a treatment for pulmonary tuberculosis for the past six months. On evaluation, the child has improved lung function, negative sputum smear, clear lungs on chest X-ray, and resolved symptoms. What is the key indicator of the success of treatment in this case?
- A. Improved lung function
 - B. Sputum smear negative
 - C. Infiltration on chest X-ray
 - D. Resolved symptoms
- ANSWER: B
55. A pediatric nurse is caring for a 10-year-old child admitted to a hospital with a diagnosis of pulmonary tuberculosis. The child had been taking anti-tuberculosis drugs and finished the treatment course; however, the AFB result is positive. The pediatric nurse is considering re-diagnosis using the TB case definition. What is the most appropriate case definition for this case?
- A. New case
 - B. Relapse case
 - C. Treatment failure
 - D. Loss to follow up
- ANSWER: C

56. An eight-month-old child was brought to an under-five OPD with complaints of fever and cough of two days duration. On assessment, the child has small, irregular red spots with a bluish-white center in his mouth, redness of the eye, and an unknown immunization status. What is the most indicative clinical feature of measles in this case?
- A. Conjunctivitis
 - B. Cough
 - C. Fever
 - D. Red spots
- ANSWER: D
57. An 11-year-old child was brought to hospital with complaints of severe muscle spasms and pain of two days' duration. The child's parents reported that the symptom started a few days after sustaining a laceration wound following a fall from a bicycle. The child's immunization status is incomplete. What is the most appropriate initial nursing intervention for this child?
- A. Administering anti-pain
 - B. Control or eliminate stimulation
 - C. Maintaining an adequate airway
 - D. Monitoring an IV infusion
- ANSWER: C
58. A two-year-old child was brought to a health center with complaints of cough and low-grade fever of two days duration. On assessment, the child has a history of recurrent respiratory infection, cough is rapid, followed by vomiting and worsens at night, flushed cheeks, eyes bulge, and protruded tongue while coughing. What is the most likely diagnosis in this case?
- A. Pneumonia
 - B. Pertussis
 - C. Diphtheria
 - D. Croup
- ANSWER: B
59. A pediatric nurse is caring for a five-year-old child who was brought to a pediatric clinic with complaints of cough and fever for three days. On assessment, the child has conjunctivitis, erythematous skin rash that starts to erupt on the face and gradually spreads downward, a brownish appearance, and fine desquamation. The child has a temperature of 38 degrees centigrade and a respiratory rate of 42 breaths/ min. What is the most likely diagnosis of this child's clinical presentation?
- A. Rubella
 - B. Poliomyelitis
 - C. Diphtheria
 - D. Measles
- ANSWER: D
60. A five-year-old male child came to the emergency department with uncontrolled bleeding after circumcision. On assessment, he is pale and his extremities are cold, and he has clammy skin. What is the most likely finding that supports the child is in shock?
- A. Decreased heart rate
 - B. Increased body temperature
 - C. Decreased blood pressure
 - D. Decreased respiratory rate
- ANSWER: C

61. A twelve-month-old male child came to the emergency department with a complaint of a frequent barking cough for a three-day duration. On assessment, he has marked sternal retractions and cyanosis when agitated. A nurse suspected the child had croup. What is the most likely additional symptom/sign that supports the diagnosis in this case?
- A. Runny nose
 - B. Nasal flaring
 - C. Chest indrawing
 - D. Inspiratory stridor
- ANSWER: D
62. A 13-year-old female adolescent was brought to the emergency department by her parents after drinking malathion for 30 minutes. On examination, she is salivating, vomiting, and becomes dizzy. What is the most likely initial management for the above scenario?
- A. Decontamination
 - B. Atropine sulfate
 - C. Activated charcoal
 - D. IV fluid
- ANSWER: A
63. A nine-year-old child was rescued from a swimming pool and brought to an emergency department immediately. The child was unconscious and had no breathing. What is the most appropriate initial management for this child?
- A. Provide oxygen
 - B. Perform CPR
 - C. Perform abdominal thrust
 - D. Monitor vital signs
- ANSWER: B
64. A four-year-old child was brought to a hospital with a sudden onset of a cough of two hours duration. As assessment, the child has hyperinflation and unilateral wheezing of the right lung, is unable to cough or make vocalizations, has a loss of consciousness, and turns cyanotic. The child has a temperature of 37.4 degrees centigrade; respiratory rate of 45 breaths /min. A pediatric nurse ruled out foreign body aspiration. What is the most appropriate initial management in this case?
- A. Put on oxygen therapy
 - B. Apply the Heimlich maneuver
 - C. Administering IV fluid
 - D. Administering antibiotics
- ANSWER: B
65. A five-year-old child has been taken to a pediatric emergency department after ingestion of an unknown substance and is suspected of having been poisoned three hours ago. On examination, the child has nausea, vomiting, pallor, and jaundice. The child weighs 15 kg and temperature of 37.5 degrees centigrade. What is the most likely causative agent for this clinical presentation?
- A. Acetaminophen
 - B. Benzodiazepines
 - C. Narcotics
 - D. Salicylates
- ANSWER: A

66. A 16-year-old female adolescent came to the hospital after ingesting bleach six hours ago. On assessment, she has a history of three episodes of vomiting and diarrhea. Otherwise, all vital signs are in normal range.
What is the appropriate management in this case?
- A. Emptying gastric with lavage
 - B. Administer activated charcoal
 - C. Secure IV line and refer for surgical review
 - D. Give Syrup of ipecac to initiate vomiting
- ANSWER: C
67. A five-year-old male child sustained a scald burn injury two hours ago. On assessment, a nurse noticed blisters formed and painful wounds on the anterior and posterior left thigh. The skin color on the burned area changed to pink in some areas and dark in others.
What is the most likely degree of burn in this case?
- A. First
 - B. Second
 - C. Third
 - D. Fourth
- ANSWER: B
68. An eight-year-old child was brought to a pediatric emergency department with a burn injury after accidentally touching a hot iron three hours ago. On examination, the child has sustained a burn injury on his forearm, and the wounds are painful, moist, red, and blistered. The child is conscious and stable.
What is the most appropriate classification of burn in this case?
- A. 1st degree burn
 - B. 2nd degree burn
 - C. 3rd degree burn
 - D. 4th degree burn
- ANSWER: B
69. A four-year-old child was brought to an under-five OPD with a burn injury two hours before. On assessment, the child sustained a hot-liquid burn injury and suffered from a full-thickness burn injury on hands, chest, and legs, covering 30 percent of the total body surface area. The child's weight is 18 kg. The child is crying and in distress.
What is the total volume of fluid needed to resuscitate the first eight hours using the Parkland formula in this case?
- A. 540 ml
 - B. 1080 ml
 - C. 2160 ml
 - D. 2700 ml
- ANSWER: B

70. A four-year-old male child, who sustained scald burn injury, was referred from a nearby health center. On assessment, his burned body surface area is 20percent and his current weight is 10 kg. A ward physician ordered fluids based on the Parkland formula.
What is the amount of fluid needed for the first twenty-four hours?
A. 400 ml and daily maintenance fluid
B. 800 ml and daily maintenance fluid
C. 1200 ml and daily maintenance fluid
D. 1600 ml and daily maintenance fluid
ANSWER: B
71. A pediatric nurse is caring for a four-year-old child who is admitted to a burn unit with scald burn injury. On assessment, the child has sustained this injury while trying to reach a hot substance placed on the table, which was accidentally pulled over the chest. The child's skin appears white with less erythema, diminished sensation, and capillary refill is prolonged.
What is the most appropriate classification of the burn injury in this case?
A. Superficial-Thickness Burn
B. Superficial partial-thickness burn
C. Deep partial-thickness burn
D. Full-thickness burn
ANSWER: C
72. A two-month-old infant born to a 48-year-old mother was presented to a pediatric OPD with chief complaints of poor feeding and abdominal distention. The infant is born at 43 weeks of gestation with 5kg. The physical examination finding shows cold extremities, dry and whitish skin, and a large posterior fontanel. The infant is diagnosed with congenital hypothyroidism.
What is the most likely risk factor in this case?
A. Mother iodine deficiency
B. Gestational age
C. Birth weight
D. Age of the mother
ANSWER: A
73. A five-year-old child is admitted to a pediatric inpatient unit for increased body weight and poor growth. Based on different investigations, the child is diagnosed with hypothyroidism.
What is the most likely expected clinical presentation in this case?
A. Normal thyroid function test, elevated thyroid-stimulating hormone, mental decline
B. Irritability, hyperactivity, short attention span, tremor, and emotional liability
C. Dry skin, puffiness around the eyes, sparse hair, constipation, and mental decline
D. Increased secretion of thyroid-stimulating hormone, hyperactivity, and constipation
ANSWER: C
74. A one-year-old child is admitted to a pediatric ward with the diagnosis of hypothyroidism. On assessment, the child has a respiratory rate of 25 breaths/min, axillary body temperature of 37.8 degrees centigrade, has poor feeding habits, weight for age is less than -3, height for age is between -3 and -2, and is unable to sit without support.
What is the most likely nursing diagnosis in this case?
A. Pain related to the disease process
B. Imbalanced energy related to poor feeding
C. Activity intolerance related to high metabolic demand
D. Delayed development related to the underlying disease
ANSWER: D

75. A five-year-old diabetic child came to a pediatric unit with his mother for a regular follow-up. A nurse requested urine analysis for ketone, and it was found to be positive for ketones. What is the most likely education the nurse should give to the mother?
- A. Advice to provide comfort and rest for the child
 - B. Encourage the child to drink low-calorie liquids
 - C. Hold the next dose of insulin
 - D. Administer a high dose of insulin now
- ANSWER: B
76. A nine-year-old child is presented with vomiting and decreased level of consciousness. Based on the laboratory result and physical examination, the child is diagnosed with diabetic ketoacidosis. A nurse is providing the ordered dose of insulin for the child. What is the most appropriate nursing intervention during the administration of insulin?
- A. Provide continuous subcutaneous insulin drips
 - B. Continue IV insulin until metabolic acidosis is corrected
 - C. Give subcutaneous short-acting insulin before meals are tolerated
 - D. Give an IV bolus of 0.1 unit/kg regular-acting insulin with intermediate-acting insulin
- ANSWER: B
77. A two-month-old infant started to have irritability, vomiting, and poor feeding after three days of admission to a pediatric ward for the follow-up of infection to tissue injury in the scalp. The physical examination revealed neurologic deficits, seizures, and Motor weakness. Based on the findings, the child is diagnosed with encephalitis. What is the most likely risk factor for the diagnosis?
- A. Developmental malformations
 - B. Local infection
 - C. Hospitalization
 - D. Trauma
- ANSWER: B
78. A 12-year-old child is admitted to a pediatric inpatient unit with the diagnosis of encephalitis. A nurse is doing a neurological assessment for the child. What is the most likely finding that supports neurologic symptoms in this case?
- A. Dysphasia and visual field defects
 - B. Decreased deep tendon reflexes
 - C. Hyperactive and alert
 - D. Memory loss and delirium
- ANSWER: A
79. A three-year-old child has been admitted to a pediatric inpatient unit with the diagnosis of bacterial meningitis. A nurse assessed the child for having fever, being lethargic, irritable, having projectile vomiting, and the head and neck were hyper-extended. Vital signs: Respiratory rate: 36 breaths/min, Pulse rate: 110 beats/min, and temperature: 40 degrees centigrade. What is the most priority nursing diagnosis in this case?
- A. Impaired Physical Mobility related to prolonged bed rest.
 - B. Hyperthermia related to the infectious process
 - C. Risk for Imbalanced Fluid Volume related to fever
 - D. Acute Pain related to meningeal irritation
- ANSWER: B

80. A 10-year-old child is admitted with complaints of severe headache and fever. Physical examinations and laboratory investigations indicated bacterial meningitis, and the child started antibiotic treatment immediately.
What is the most likely nursing intervention for the patient?
- A. Administer anti-pain and respiratory isolation for at least 24 hours
 - B. Administer Antibiotics and respiratory isolation for at least 24 hours
 - C. Administer Antibiotics and elevate the bed with two pillows
 - D. Administer anti-pain and a high volume of intravenous fluid
- ANSWER: B
81. A four-year-old child is diagnosed with a seizure and has been treated. Once the child finishes all the treatments needed, a nurse perceived that the parents did not understand what they had to do if the child experiences a seizure again, and the nurse planned to teach them what to do.
What is the most appropriate information the nurse shared with the parents?
- A. Restrict Movement to Decrease Injuries
 - B. Take the child to a health institution while the child is seizing
 - C. Put the child in the lateral position and wait until the seizure stops
 - D. Fire matches and bring them to the child's nose to stop the seizure
- ANSWER: C
82. A six-year-old child is admitted to the pediatric inpatient ward with the diagnosis of encephalitis. On assessment, the child has had two episodes of seizure and delirium since admission. A nurse planned to give health education on the risk of injury to the parents.
What is the most likely nursing intervention to decrease the risk of injury?
- A. Reorient the patient frequently
 - B. Monitor input and output
 - C. Counsel on how to ambulate
 - D. Counsel to maintain a quiet environment
- ANSWER: D
83. A five-year-old child is admitted to a pediatric emergency department with a history of falling accident. The child has tenderness and bruising on the overlying skin of the lower extremity. Based on a detailed assessment, a nurse suspected for fracture.
What additional clinical presentation most likely confirms the diagnosis?
- A. Bleeding and swelling of the affected extremity
 - B. Limited range of motion of the affected extremity
 - C. Decreased function and crepitus were heard from the site
 - D. Edema and pain, fever at the site
- ANSWER: C
84. A nurse is taking care of an 8-year-old child at a pediatric emergency unit for a fall. The diagnosis confirmed an open fracture of the right lower leg. On nursing assessment, the child has Pain at the site of injury, swelling, bleeding, and deformity.
What is the most likely priority nursing care for this child?
- A. Preventing wound infection
 - B. Decrease pain
 - C. Manage for bleeding
 - D. Manage the deformity
- ANSWER: C

85. A 12-year-old child is admitted to a pediatric inpatient unit with the diagnosis of chronic osteomyelitis and scheduled for a bone graft procedure in the operating room. What is the most likely preoperative nursing care in this case?
- A. Neurovascular status assessment
 - B. Watch for hemorrhage
 - C. Stabilize musculoskeletal structures
 - D. Check hydration, protein, and caloric intake
- ANSWER: D
86. A two-year-old child is diagnosed with developmental dysplasia of the hip. On physical examination, the hip is deviated from the normal, with limitation of abduction; moreover, the level of the pelvis is unequal. What is the most likely complication in this case?
- A. Joint pain
 - B. Osteoarthritis
 - C. Transient synovitis
 - D. Necrosis of the femoral head
- ANSWER: D
87. A six-year-old child is presented to a pediatric outpatient unit with complaints of swelling and pain in the ankle joint. On assessment, the child has painful limitation of range of motion, overlying skin warmth and redness, body temperature of 38 degrees centigrade, and white blood cell count (WBC) and C-reactive protein (CRP) are elevated. Aspiration of joint fluid culture shows Gram-positive. What is the most likely medical diagnosis for the child?
- A. Transient synovitis
 - B. Osteomyelitis
 - C. Septic arthritis
 - D. Pyomyositis
- ANSWER: C
88. A one-month-old infant was brought to a pediatric OPD with a complaint of inability to breastfeed for three days. As an assessment, the baby has persistent vomiting, yellowish discoloration of the eyes, skin, palms, and soles. What is the most appropriate finding to confirm the classification of severe jaundice based on the IMNCI protocol?
- A. Discoloration of the eye and skin
 - B. Discoloration of the palms and soles
 - C. Inability to breastfeed
 - D. Persistent vomiting
- ANSWER: B
89. A 40-day-old child was brought to a hospital with complaints of diarrhea and inability to feed for 15 15-day duration. On assessment, skin Pinch goes back slowly, and the child had irritability and no blood in the stool examination. A pediatric nurse classified the child with severe, persistent diarrhea. What is the most likely assessment finding used to confirm the classification in this case?
- A. Being irritable
 - B. Consistency of the stool
 - C. Duration of diarrhea
 - D. Slow skin Pinch
- ANSWER: C

90. A two-month-old infant was presented to an OPD with a complaint of watery diarrhea five times per day that lasted for a three-day duration. On assessment, the infant is restless, has sunken eyes, and the skin pinch goes back slowly. A pediatric nurse assessed that the child has dehydration. What is the appropriate classification of dehydration based on the IMNCI protocol?
- A. Moderate dehydration
 - B. No dehydration
 - C. Severe dehydration
 - D. Some dehydration
- ANSWER: D
91. A two-month-old infant was brought to a pediatric OPD with complaints of oral thrush and inability to breastfeed of seven days duration. On assessment, the child's mother is HIV positive and on anti-retroviral therapy. The child is on breastfeeding, the DNA PCR test for the baby is negative, but the HIV antibody test is positive. What is the most appropriate classification of this infant based on the IMNCI protocol?
- A. HIV exposed
 - B. HIV infected
 - C. HIV infection unlikely
 - D. HIV status unknown
- ANSWER: A
92. A two-month-old child has been brought to a pediatric OPD with a complaint of diarrhea of 15 days duration. On assessment, the infant is restless, irritable, has sunken eyeballs, skin pinch goes back slowly, and no blood is found in the stool examination. What is the most likely treatment for this infant according to the IMNCI protocol?
- A. Treat diarrhea at home with ORS
 - B. Treat the infant with option B
 - C. Oral Amoxicillin for 5 days
 - D. Ciprofloxacin for 3 days
- ANSWER: B
93. A two-month-old infant was brought to a health center with a complaint of diarrhea three times per day for a day's duration. On assessment, the infant has moist mucosa, is well breastfed, skin pinch goes back within less than two seconds, and weighs 5kg. A nurse assessed and confirmed that diarrhoea with no dehydration and treated with ORS. How many mL of ORS is required in addition to home fluid to treat this baby?
- A. 300ml
 - B. 400ml
 - C. 50ml
 - D. 500ml
- ANSWER: C
94. A one-month-old infant was presented to a pediatric OPD with a complaint of diarrhea four times per day for a day's duration. On assessment, the baby is stable; feeding well, has skin that goes back within less than two seconds, and has moist mucosa. What must be prioritized in counselling the mother on the treatment of this neonate?
- A. Antibiotics for diarrhea
 - B. Fluids and Zinc supplement
 - C. Formula feeding immediately
 - D. Supplementary mixed feeding
- ANSWER: B

95. A three-day-old neonate was brought to a pediatrics clinic with a complaint of yellow pigmentation of the eye for a day. On assessment, the baby was born at 38 weeks of gestation, temperature of 36.7 degrees centigrade, was a well-breastfed baby, and had light yellowish discoloration of the eye but not the skin, palms, and soles involved.

What is the most appropriate counselling that the nurse needs to deliver to this mother?

- A. Exposure to natural light daily
- B. The baby needs phototherapy
- C. To discontinue breastfeeding
- D. Warm the infant by skin-to-skin contact

ANSWER: A

96. A three-year-old child was brought to a pediatric clinic with a complaint of a cough of two days' duration. On assessment, the child has chest in drawing, oxygen saturation of 86 percent, and respiratory rate of 44 breaths/ min. A pediatric nurse suspected that the child was suffering from severe pneumonia.

What is the most likely sign that made the nurse suspicious about the child's condition?

- A. Chest in drawing
- B. Cough
- C. Oxygen saturation
- D. Respiratory rate

ANSWER: A

97. A Four-year-old child was brought to a pediatric OPD with a complaint of ear pain for a three-day duration. On assessment, the child has tender and palpable swelling behind the left ear, fever, and mild cough, but no ear discharge. A pediatric nurse classified the child as having mastoiditis.

What is the most likely sign used to classify the child's condition?

- A. Absence of ear discharge
- B. Ear pain for a three-day duration
- C. Tender swelling behind the ear
- D. Fever and mild cough

ANSWER: C

98. A seven-month-old child has been brought to a health centre with a complaint of fever for a day's duration. On assessment, the infant has projectile vomiting after feeding, persistent fever, temperature of 38 degrees centigrade, and respiratory rate of 40 breaths /min, bulging of anterior fontanel, and stiff neck. However, the child has a negative blood film test for malaria but no recent travel history to malaria risk areas.

What is the most appropriate classification of this case according to the IMNCI protocol?

- A. Fever with malaria
- B. Fever with no malaria
- C. Very severe febrile disease
- D. Severe complicated measles

ANSWER: C

99. A four-month-old child was brought to a health center for routine immunization. On assessment, the infant follows objects with eyes past midline, responds with sounds when you speak and laughs out loud, holds an object with one hand, and lifts head and chest when put on tummy. What is the most appropriate developmental classification of this infant based on the IMNCI protocol?

- A. Definite developmental delay
- B. Probable developmental delay
- C. No developmental delay
- D. Suspected developmental delay

ANSWER: C

100. A five-year-old child was brought to a health center with a complaint of severe ear pain of five days duration. On assessment, the child has pus draining from the left ear, loss of appetite, temperature of 37.3 degrees centigrade, and respiration rate of 30 breaths/ min. There is no swelling behind the ear. What is the most appropriate classification of this child's case based on the IMNCI protocol?

- A. Otitis media
- B. Acute ear infection
- C. Chronic ear infection
- D. Mastoiditis

ANSWER: B

101. A Five-year-old child was brought to a pediatric OPD with complaints of cough and fever of two days duration. On assessment, the child has fever, fast breathing; oxygen saturation is 92percent, and dry cough. Otherwise, the child has no ear drainage or skin pustule. What is the most appropriate management of the child's condition?

- A. Advise the mother on better home care for the baby
- B. Give Amoxicillin for 5 days
- C. Refer urgently to the hospital
- D. Soothe the throat with a safe remedy

ANSWER: B

102. A five-year-old child was taken to a health center with a complaint of an earache for more than two weeks. On assessment, the child has pus discharge from both ears, a temperature of 36.5 degrees centigrade, a pulse rate of 70 beats/min, and a respiratory rate of 26 breaths/ min. What is the most appropriate treatment for this child according to the IMNCI protocol?

- A. Amoxicillin for 5 days
- B. Quinolone for 14 days
- C. Ceftriaxone IV for 07 days
- D. Chloramphenicol IV for 05days

ANSWER: B

103. A three-day-old term infant was brought to an under-five clinic by his mother with complaints of difficulty feeding and sleeping immediately after feeding for the last two days. A pediatric nurse wants to assess whether or not the mother usually puts the baby in a good position. The nurse instructed the mother to feed the baby.

What is the most likely positioning the nurse may observe?

- A. Mouth wide open
- B. Chin touching the nipple
- C. Upper lip turned outward
- D. The whole body is unsupported

ANSWER: D

104. A four-month-old term male neonate who was on exclusive breastfeeding was brought to an under-five OPD by his mother with a complaint of poor feeding, failure to grow, and irritability while in contact with his mother's breast for one week. The mother reported that on average, she feeds her child about 10 times per day. The birth weight of the baby was 3000 g, but at the time of assessment, the baby's weight was 3500 g.

What is the likely history that the nurse should ask the mother?

- A. How frequently did you feed the baby in the last week?
- B. Do you empty one breast before switching to the other?
- C. Do you increase the frequency of breastfeeding during illness?
- D. Does the infant receive any other food or drink?

ANSWER: B

105. A Nine-month-old female infant was brought to an EPI clinic by her mother for vaccination. She started complementary feeding at the age of six months by giving 4-6 tablespoons of food 3- 5 times a day. She prepared and enriched porridge made of cereal, shiro fitfit, merkefitfit, mashed potatoes and carrot, mashed gomen, eggs, and fruits by adding butter and oil every time.

What is the most appropriate feeding practice by the mother?

- A. Early initiation of breastfeeding
- B. Increased frequency of diversified foods
- C. Timely initiation of complementary foods
- D. Increased frequency of meals during illness

ANSWER: C

106. A four-year-old male child was admitted to a pediatric ward with the diagnosis of complicated severe acute malnutrition. He started the initial treatment accordingly.

What is the most important physical finding that supports the diagnosis?

- A. Brittle hair
- B. MUAC of 11cm
- C. Ineffective feeding
- D. Bilateral pitting oedema of grade III

ANSWER: D

107. A three-year-old male child was presented to an under-five with complaints of diarrhea and poor appetite for the last 3 days. On physical examination, the infant looks acutely sick. Anthropometric measurement; WFA less than -2, MUAC, 12cm, height-for-age (HFA) falls below 2 SDs.

What is the most likely measurement that indicates the baby is stunted?

- A. Sick looking
- B. Low weight for age
- C. low height-for-age
- D. Low mid-upper arm circumference

ANSWER: C

108. While caring for a two-year-old child who was admitted with severe acute malnutrition, a duty nurse started assessment and found Vital signs: Respiratory Rate, 32 breaths/min; temperature, 34.4 degrees centigrade; and pulse rate of 94 beats/min. The random blood sugar is 76mg/dl.

What is the most likely priority nursing diagnosis in this case?

- A. Ineffective thermoregulation
- B. Imbalanced nutrition: less than body requirements
- C. Risk for unstable blood glucose levels
- D. Imbalanced energy field

ANSWER: A

109. A four-year-old child was presented to a pediatric OPD with complaints of watery diarrhea and loss of appetite for a day's duration. On assessment, a nurse revealed a respiratory rate of 44 breaths/min, temperature 38-degree centigrade, distended abdomen, yellow discoloration of eyes, and bilateral pitting edema.

What is the most likely finding to diagnose malnutrition in this case?

- A. Discoloration of the eye
- B. Distended abdomen
- C. Bilateral pitting oedema
- D. Increased temperature

ANSWER: C

110. A 34-month-old infant was presented to a pediatric emergency department with a chief complaint of being unable to feed for a day. On physical examination, he looks acutely sick but conscious. The respiratory rate of 32 breaths/min and temperature of 36 degrees centigrade are recorded. The random blood sugar of the child reads 30mg/dl. A nurse thoroughly evaluated and diagnosed the child with complicated severe acute malnutrition.

What is the most appropriate nursing management for the child?

- A. Securing the IV line and giving 50ml of 10 percent glucose
- B. Inserting NGT and providing 50ml of 10percent glucose
- C. Keep the vein open and give 50ml of 40percent glucose
- D. Treat associated hypothermia

ANSWER: B

111. A 10-month-old infant was presented at the health center with poor weight gain of 2weeks duration. The mother said she has the desire to eat, but has not gained weight since two weeks back. Otherwise, she has no edema, diarrhea, vomiting, or cough. On physical examination, she is alert, vital signs: Respiratory Rate-36 breaths/min, Temp, 36.6 degrees centigrade, random blood sugar, 68mg/dl. Anthropometric measurements are recorded as follows: MUAC of 12cm, WFH of 3 score.

What is the most appropriate nursing management in this case?

- A. Provide F-75
- B. Provide Plumpy Nut
- C. Provide plumpy' sup
- D. Provide diluted F-100

ANSWER: C

112. A nine-month-old female infant accompanied by her mother came to the immunization unit for the measles vaccine. While plotting the growth monitoring chart, a nurse recognized that the infant was not gaining weight adequately.

What is the most appropriate nursing management in this case?

- A. Counsel the mother to increase the frequency of feeding during illness
- B. Praise the mother and provide the next vaccination upon appointment
- C. Counsel the mother to feed her baby on a diversified diet
- D. Counsel the mother on an appropriate weaning strategy

ANSWER: C

113. A 20-day-old preterm baby was on cup feeding of 20 ml of breast milk every two hours. The baby sucks well but is unable to swallow and starts vomiting on every feeding. What is the most appropriate nursing management in this case?
- A. Insert an NGT to feed the baby
 - B. Hold feeding and put on fluids
 - C. Make the feeding 10ml every 1 hour
 - D. Change breast milk to formula milk.
- ANSWER: A
114. A Four-year-old male child was brought to a pediatric clinic by his mother with complaints of loss of appetite and bilateral leg swelling for a week's duration. On assessment, body temperature of 36 degrees centigrade, respiration rate of 42 breaths/min, cold extremities, and capillary refill after five seconds. Also, his MUAC was 11cm, and bilateral pitting edema extended to his knees was noted. What is the most likely complication the child developed?
- A. Shock
 - B. Hypothermia
 - C. Anaemia
 - D. Pneumonia
- ANSWER: A
115. A three-year-old girl was admitted to a pediatric ward with the diagnosis of complicated severe acute malnutrition since yesterday. The blood sugar level of the child is 44mg/dl. A nurse provided 10percent dextrose solution and F-75. What is the most likely improvement indicator after two hours?
- A. reduction of oedema
 - B. Weight increases by 1kg
 - C. Urine output of 20ml/hour
 - D. Blood sugar greater than 54mg/dl
- ANSWER: D
116. A two-year-old male whose weight is 9kg is admitted to a pediatric therapeutic feeding with the diagnosis of complicated severe acute malnutrition since last week. After four days of F-75 treatment, the child fulfilled the criteria to start F-100. After one day on F-100 treatment, a pediatrician decided to reverse the child's feeding to F-75. What is the most likely criterion to reverse his feeding?
- A. Decrease in oedema
 - B. Infection resolution
 - C. Improve in appetite
 - D. Increase in weight by 12g/kg/day
- ANSWER: D
117. A three-year-old boy was brought to a pediatric clinic with complaints of weight loss and loss of appetite for a week's duration. He has no history of cough, fever, or body swelling. On examination, prominent zygomatic bone, sunken eye, and skin and bone appearance were noted. His weight is 10 kg and his height is 100cm. The normal weight of children of the same height is 20 kg. What is the most likely diagnosis in this case?
- A. Severe wasting
 - B. Severe stunting
 - C. Severe underweight
 - D. Chronic malnutrition
- ANSWER: A

118. A mother brought her 3-year-old female child to a pediatric outpatient department with complaints of shortness of breathing and whitish discoloration of the palm for a two-week duration. Her respiratory rate is 66 breaths/ min and body temperature of 35.7 degrees centigrade. Laboratory result reveals WBC=5000 cells/mm³, hemoglobin 6.4gm/dl, and platelet count of 185,000cells/mm³. What is the most likely diagnosis of this child?
- A. Septic shock
 - B. Heart failure
 - C. Severe anaemia
 - D. Severe Pneumonia
- ANSWER: C
119. A four-year-old male child was brought by his mother from an internally displaced camp with a complaint of diarrhea for a day's duration. On assessment, a pediatric nurse noted sunken eyes, skin that pinches back slowly, and skin folds on the buttocks and thighs. The child is admitted to a stabilization center for F-75 and F100 treatment. What additional relevant finding supports diagnosing this child?
- A. Abdominal distention
 - B. Nausea and vomiting
 - C. Skin and bones' appearance
 - D. Non-pitting oedema on both extremities
- ANSWER: C
120. A mother of a four-month-old infant came to the hospital with a complaint that her breast had not been secreting any milk for the last two days. On assessment, a nurse noticed no milk secretion from the breast while squeezing her nipples. What is the most likely advice that has to be given to the mother?
- A. Give formula milk
 - B. Advise to start cow milk
 - C. Advise her to start a semisolid diet
 - D. Apply cold compression over her breast
- ANSWER: A
121. A five-year-old female child was brought to a pediatric clinic by his mother with complaints of diarrhea and vomiting for a week. On assessment, a pediatric nurse noted a change in skin and bone appearance and WFH measurement score of less than -3. Based on the findings, the child was admitted to a stabilization center. What is the priority nursing intervention in the above scenario?
- A. Promoting optimal nutrition
 - B. Provide frequent oral hygiene
 - C. Withholding Food to rest the GI tract
 - D. Advice to restrict intake of high-calorie calories
- ANSWER: A
122. A two-year-old child was brought to a pediatric clinic with a complaint of diarrhea for the last three days. On assessment, a pediatric nurse noted weight for height less than -3 Z score, MUAC=11cm, and grade II bilateral pitting edema and passed the appetite test. What is the most likely diagnosis in this case?
- A. Moderate acute malnutrition
 - B. Uncomplicated chronic malnutrition
 - C. Complicated severe acute malnutrition
 - D. Uncomplicated severe acute malnutrition
- ANSWER: D

123. A three-year-old female was admitted with the diagnosis of complicated severe acute malnutrition. The child was managed by using F-75 and F-100 for the last three weeks. Currently, the child's condition is stable and ready for discharge.
What criterion has to be fulfilled to discharge this child?
- A. MUAC is greater than or equal to 11.5 cm
 - B. Weight-for-age greater than or equal to 2 Z-score
 - C. No oedema observed in one visit
 - D. Weight-for-height greater than or equal to 2 Z-score
- ANSWER: D
124. A nurse assessing a newborn infant after five minutes of delivery found that the baby has slightly flexed hands, tone, a heart rate of 70 beats/min, blue discoloration extremities, and a respiratory rate of 30 breaths/min.
What is the most likely critical finding that informs the condition of the infant?
- A. Slightly flexed hand muscle tone,
 - B. Heart rate of 70 beats/min
 - C. Blue discoloration of the extremities
 - D. Respiratory rate of 30 breaths / Min.
- ANSWER: B
125. A woman in active labor came to a health center at 38 weeks of gestation. The membranes ruptured shortly after arrival, and the amniotic fluid was found to be meconium-stained. At the time of birth, the baby appears to be term, extremities flexed slightly, and not crying.
What is the most appropriate immediate step to be applied to this baby?
- A. Clap the cord, cut, and put the baby on the mother's chest
 - B. Putting to skin-to-skin on the mother's chest and covering with a warm blanket.
 - C. Holding the baby in a warm blanket, drying and stimulating the baby to breathe
 - D. Clap the cord and cut, and take the baby to the radiant warmer
- ANSWER: C
126. A 38-year-old pre-eclamptic-prim gravid woman arrives at an emergency room having a precipitated labor at 33 weeks of gestation, and the membrane is ruptured very shortly after arrival. At the time of birth, the baby appears small for gestational age and has poor muscle tone. A nurse anticipates that the baby needs resuscitation based on clinical presentation and risk factors.
What is the most likely risk factor that necessitates neonatal resuscitation?
- A. Advanced age of the mother
 - B. Pre-eclampsia of the mother
 - C. Prematurity of the newborn
 - D. Poor muscle tone of the newborn
- ANSWER: C
127. A nurse in the labor ward is assisting emergency cesarean sections for the indication of fetal distress. The nurse notes that the woman is, 41-year-old, multi-para, at 43 weeks of gestation, and has meconium-stained show.
What is the most likely physical finding of this newborn just after birth?
- A. Pink, thick skin, with lanugo, and vernix
 - B. Pale, thin skin with lanugo and vernix
 - C. Pale thin skin, with wrinkles and peeling skin
 - D. Pale, thick skin with meconium staining
- ANSWER: C

128. A full-term, big for gestational age, born just 30 minutes ago, is admitted to the neonatal intensive care unit with unstained suckling, weak moro reflex, lethargic and hypotonic, irregular respiration. Apgar score was 3 at 1st minute and 6 at 5th minute.

What clinical presentation most likely describes the neonate?

- A. Hyaline membrane disease
- B. Perinatal asphyxia
- C. Meconium aspiration syndrome
- D. Neonatal distress syndrome

ANSWER: B

129. A newborn baby born by cesarean section at 43 weeks of gestation for an indication of prolonged labor and reassuring fetal heartbeat was brought to the NICU with a diagnosis of large for gestational age and respiratory distress. Apgar scores were 3 and 5 within the first and five minutes of delivery. A nurse noticed a respiratory rate of 69 breaths/min, intercostal and sternal retractions, expiratory grunting, cyanotic lips, and anteroposterior diameter of the chest.

What is the most likely diagnosis in this case?

- A. Prenatal asphyxia
- B. Transient tachypnea
- C. Hyaline Membrane Disease
- D. Meconium aspiration syndrome

ANSWER: B

130. A 30-hour-old baby born after 22 hours of membrane rupture and whose mother was on antibiotics, is referred from a health center with the diagnosis of full term, normal birth weight, appropriate for gestational age, but failure to suck. A nurse noticed that the baby had hypothermia, grunting, severe chest indrawing, and capillary refill time greater than 3 seconds. And laboratory results reveal neutrophils 68 percent, lymphocytes 35 percent, monocytes 3percent. CSF analysis reveals clear color, WBC 10 cells/mm³, glucose 35 mg/dl, and Protein 90 mg/dl.

What is the most likely diagnosis for the baby?

- A. Early-onset neonatal meningitis
- B. Late-onset neonatal sepsis
- C. Early-onset neonatal sepsis
- D. Late-onset neonatal meningitis

ANSWER: C

131. A 12-hour newborn is referred from a health center with the diagnosis of term, normal birth weight, appropriate for gestational age, and failure to suck. On physical examination, a nurse noticed mottled skin, a pulse rate of 190 / min, breathing rate of 68/min, body temperature of 37.7-degree centigrade, and prolonged capillary refill time. Based on the finding, the nurse decided to administer fluid.

What is the most appropriate choice?

- A. Ringer lactate
- B. Normal saline, 0.9 percent
- C. Dextrose in water, 5 percent
- D. Dextrose in water, 10 percent

ANSWER: D

132. A 36-hour-old newborn was admitted to the NICU with the diagnosis of post-term, normal birth weight, appropriate for gestational age, and prenatal asphyxia stage II. The baby is on intranasal oxygen, maintenance fluid with 1 ml/kg calcium gluconate. A nurse noticed that the child started having seizures.

What is the most preferred drug in the management of this baby?

- A. Phenobarbitone
- B. Phenytoin
- C. Diazepam
- D. Sodium valproate

ANSWER: A

133. A seven-day-old baby born to a 34-year-old mother at 40 weeks of gestation and healthy on exclusive breastfeeding is brought to the NICU with a complaint of bleeding from the circumcision site for 6 hours. On physical examination, the baby has pallor hands, delayed capillary refill time, and is sleepy, and the hematocrit result is found to be 7 mg/dl.

What is the most appropriate immediate intervention for this baby?

- A. Vitamin K 1mg IV and fresh-frozen plasma 10-20ml/kg
- B. Vitamin K 1mg IV and Normal saline 10-20ml/kg
- C. Fresh-frozen plasma 10-20ml/kg followed by platelet
- D. Fresh-frozen plasma 10-20ml/kg followed by ringer lactate

ANSWER: A

134. A 32-hour-old neonate is admitted to the NICU with a diagnosis of preterm and respiratory distress syndrome. Hence, the baby is on the third dose of antibiotic, nasal CPAP 8cm H₂O for 30 hours, and has maintained stable breathing and oxygen saturation. Unfortunately, the baby started to grunt, cyanotic, restlessness, apneic, tachypneic, and respiratory distress worsened. On physical examination, the baby has decreased air entry on the right side of the lung, hyperresonance to percussion, and the trachea is deviated to the left side. A nurse suspected a complication of CPAP.

What is the most likely complication that happened to this baby?

- A. Oxygen toxicity
- B. Aspiration pneumonia
- C. Diaphragmatic hernia
- D. Pneumothorax

ANSWER: D

135. A nurse in NICU is caring for a five-day-old baby born by instrument for an indication of delayed second stage and diagnosed as term, appropriate for gestational and birth asphyxia. A pediatrician ordered breast breastfeeding trial.

What is the most reassuring good breast attachment sign that the nurse is looking for?

- A. More areola is seen on the lower part of the breast
- B. Baby sucks frequently and swallows
- C. Lower lips everted in
- D. Chin of the baby touches the breast

ANSWER: D

136. A father presented his eighteen-month-old male child to a health center for deworming and vitamin A supplementation. On arrival, a nurse working in an EPI room assessed his anthropometric measurement of weight for age. When she checks the data with the CDC growth chart, it is less than -2SD.

What is the most likely interpretation of the finding?

- A. Normal
- B. Stunted
- C. Wasted
- D. Underweight

ANSWER: D

137. A 10-month-old baby is presented to a nearby early childhood development clinic to check his developmental milestones expected at this age. A nurse working in that department performed a detailed physical examination and took a thorough history.

What is the most likely developmental milestone expected for this child?

- A. Bubbling
- B. Sits independently
- C. Says mama or dada appropriately
- D. Says mama or dada inappropriately

ANSWER: C

138. A mother brought a 9-month-old infant to an early childhood development clinic with a complaint of being unable to sit independently like his age group, but he is alert about his brain development. A nurse working in that department performed a detailed physical examination and took a thorough history.

What is the prioritized nursing management for this child?

- A. Take the infant into the holy water
- B. Promote the mother to exercise daily
- C. Take the infant to traditional healers
- D. Keeping the child for a short period without any treatment

ANSWER: B

139. A 16-month-old infant was presented to an early childhood development clinic with a complaint of being unable to walk like his peers, but a health professional ensured that the child had no other developmental problems. After complete assessment and physical examination, the health professional advised to implement all activities ordered by him and shall monitor closely and give an appointment to come back after three months.

What is the most likely expected growth and development for this child?

- A. Become paralyzed
- B. Can walk to some extent
- C. The same as the previous one
- D. Has developed other forms of motor problems

ANSWER: B

140. A one-year-old baby boy was brought to a well-baby clinic for routine follow-up. A nurse in charge has assessed and noticed a weight of 9.2kg, height 62cm, mid-upper arm circumference 13.7cm, and concludes that the growth of the child is appropriate for his age. Yet the mother complains that he is not as active as his elder sister. The nurse confirmed that the infant is performing as expected for his age and reassured the mother.

What is the most likely confirmatory assessment finding?

- A. Uses thumb and index finger to pick up a pellet
- B. Able to walk by supporting self on furniture
- C. Sits alone for a short period.
- D. Rolls from back to stomach

ANSWER: B

141. A three-year-old child was brought to a pediatric OPD for regular cardiac follow up and the mother complains that the child is unable to gain weight despite having a good appetite. A nurse has measured a weight of 9 kg and height of 101 cm, which lies below -2Z, but above -3Z, and mid-upper arm circumference of 12.3cm.

What is the most likely interpretation of the child's anthropometric measurement?

- A. Moderate acute malnutrition
- B. Moderately underweight
- C. Severe acute malnutrition
- D. Severe underweight

ANSWER: A

142. A mother brought a three-year-old daughter to a well-baby clinic with a complaint of the annoying behavior that might require medical attention. The mother also reported that the child frequently responds to the mother's simple requests with a "no" answer. Furthermore, the child has had an increase in the number of temper tantrums at bedtime and refuses to go to bed. She is afraid her daughter will hurt herself during a temper tantrum while being hugged.

What is the most appropriate response to the mother in this case?

- A. The child should undergo some counselling, and the a need for critical follow.
- B. The behaviour is serious, and possible interventions should be explored.
- C. The mother and father should evaluate their child-rearing practices
- D. The behaviour is typical, and parents should show patience for some time

ANSWER: D

143. A Mother brought her 7-month-old female infant to a health center as she cannot manage to sit without support. Otherwise, the infant is stable and active. She had been feeding her baby breast milk only for the first 6 months, until she began to provide additional food. She had been feeding her breast on average 5 times per day.

What is the most appropriate nursing management for this infant?

- A. Inform the mother that it is an irreversible condition
- B. Counsel the mother on play and responsive caregiving
- C. Referring for the developmental delay evaluation and management
- D. Praise the mother for feeding her infant well, and advise her to keep on

ANSWER: B

144. A nurse in an under-five clinic is evaluating the progress of growth of a 12-month-old male infant using a growth chart. The nurse noticed that the weight-for-age growth curve falls below the 5th percentile.

What is the most likely nursing management in this case?

- A. Refer the infant for admission
- B. Praise the mother and send her home
- C. Counsel the mother on infant feeding
- D. Admit to outpatient therapeutic program

ANSWER: C

145. A mother brought her 15-month-old child to an EPI unit for the second dose of measles vaccination. A nurse working in the unit has been monitoring the growth of the child during each visit using a growth chart. The nurse recently recognized that the growth curve is down sloping, crossing the action line.

What is the most appropriate management for this child?

- A. Counsel on dietary diversity
- B. Refer for further evaluation
- C. Congratulations on completing the vaccination
- D. Praise the mother and appoint for a follow-up visit

ANSWER: A

146. A nurse is assessing a twelve-month-old infant with low weight for the gestational age who was brought to a pediatric clinic for growth and development follow-up. The nurse noticed that the child is developing well.

What is the most likely normal developmental milestone that the infant may exhibit?

- A. Sits up with support
- B. Waves bye-bye
- C. Walks without holding furniture
- D. Walks with one hand held

ANSWER: B

147. A four-year-old boy was brought to a pediatric clinic by his mother with a complaint of a painful red lesion over the lower lip for a days duration. On examinations, a body temperature of 37.5 degree centigrade, respiration rate of 34 breaths/min, nasal discharge, and macular transparent bullae were noted. The culture of the wound revealed staphylococci.

What is the most likely risk factor for the above case?

- A. Contact history
- B. Weak immunity
- C. Airway infections
- D. Trauma to the skin

ANSWER: C

148. An 11-year-old girl was presented to a pediatric OPD with a complaint of generalized skin rash following consumption of egg and fish. Physical findings indicated lichenification on the popliteal fossa. Her vital signs are in the normal range.

What additional clinical presentation will be noted in the above case?

- A. Open comes done
- B. Severe dry skin
- C. Threadlike burrows
- D. Honey-crusted lesion

ANSWER: B

149. A 14-year-old adolescent male visited a hospital with necrotized tissue over his right upper arm. On assessment, he had a history of a right upper arm fracture one week ago, and a cast was applied. On palpation, there is no tenderness to the affected site.

What is the priority nursing care for this patient?

- A. Providing wound care
- B. Administering intravenous fluid
- C. Performing incision & drainage
- D. Removal of the devitalized tissues

ANSWER: D

150. A nine-year-old boy was brought to a health facility by his mother with complaints of itching, maceration, and peeling of the interdigital spaces for five days. He received zinc oxide treatment for two weeks and got relieved.

What physical examination supports to diagnosis of the above case?

- A. Visualizing honey color crust
- B. Excavated threadlike burrows
- C. Visualizing a sharply demarcated satellite lesion
- D. Presence of lymphadenopathy and pitting oedema

ANSWER: B

151. A seven-year-old child was admitted to a pediatric ward due to bullous impetigo on his left hand for three days. Children of the same age with third-degree burns occupied the bed next to him.

What is the most likely strategy to prevent the spread of infection in the above case?

- A. Isolation
- B. Donning gloves
- C. Hand washing
- D. Antiseptic

ANSWER: A

152. An eight-year-old girl was brought up by her mother with complaints of itching and pain in the head for a week. On assessment, a nurse noted hair loss and swelling of the lymph node on the lateral side of her neck. Laboratory results revealed positive KOH. Based on the findings, griseofulvin is ordered for two months.

What information has to be given to increase the absorption of the medication?

- A. Increase daily fat intake
- B. Increase daily protein intake
- C. Increase carbohydrate intake
- D. Decrease daily protein intake

ANSWER: A

153. A 10-year-old child who visited a hospital with a diagnosis of impetigo received amoxicillin three weeks ago. A caregiver reported that the child's progress was poor, and he has cola-colored urine and increased frequency of urination. Laboratory analysis reported a red blood cell count of 20 and proteinuria.

What is the most likely complication the above child developed?

- A. Acute renal failure
- B. Nephrotic syndrome
- C. Acute rheumatic fever
- D. Poststreptococcal nephritis

ANSWER: D

154. A 14-year-old girl, well known atopic dermatitis patient, came to the hospital for her follow-up. Now her previous medication has been changed to diphenhydramine for a one-month duration. What is the most expected improvement to be seen after the administration of this drug?
- A. Decrease in Skin pruritus
 - B. Decrease in skin dryness
 - C. Decrease in skin integrity
 - D. Decrease risk of infection
- ANSWER: A
155. A 21-day-old female neonate was brought to a pediatric clinic by her mother with a complaint of a red and painful lesion on her genital area for five days duration. Upon assessment, erythematic, sharply demarcated, and satellite lesions are seen over her genital area. What is the most likely diagnosis of the above case?
- A. Cellulites
 - B. Dermatitis
 - C. Candidosis
 - D. Tinea cruris
- ANSWER: C
156. A mother brought her two-month-old infant to a pediatric clinic with a complaint of painful and red lesions around the genital area for two days. On assessment, a pediatric nurse revealed erythema and satellite lesions with sharply demarcated margins. What is the first line of therapy for the above case?
- A. Prednisolone PO
 - B. Amoxicillin syrup
 - C. Imidazole cream
 - D. Hydrocortisone cream
- ANSWER: C
157. A seven-year-old male school-age child was admitted to a pediatric ward with the diagnosis of chronic atopic dermatitis. On assessment, a nurse noted intense itching and excoriated lesions on the upper arm, including his elbow. Vital signs and laboratory results are in the normal range. What is the appropriate dressing recommended in the above case?
- A. Dry dressing
 - B. Wet dressing
 - C. Alginate dressing
 - D. Oil-based dressing
- ANSWER: B
158. A mother brought her infant for routine vaccination to an immunization clinic. According to the schedule, a pediatric nurse decided to administer the inactivated polio vaccine. What should be the most appropriate age for the infant to receive the vaccine?
- A. At 6 weeks
 - B. At 10 weeks
 - C. At 12 weeks
 - D. At 14 weeks
- ANSWER: D

159. A pediatric nurse visited a family displaced from their residence at their immigration center to provide vaccination for the target community. The nurse found a 12-month-old child who had not received his first dose of PCV vaccine, and the nurse administered the first dose of PCV vaccine. What should be the minimum age of the child to administer the second dose of PCV?
- A. At 14 months
 - B. At 16 months
 - C. At 18 months
 - D. At 24 months
- ANSWER: C
160. Pediatric nursing students are practicing their final attachment in a community. As part of their community practice, they planned to give the human papillomavirus (HPV) vaccine at the elementary school to prevent associated risks in the female students. Based on their plan, they administered the first dose of the vaccine to all eligible female students. What is the recommended period to provide the next dose of the vaccine?
- A. After 4 months
 - B. After 6 months
 - C. After 7 months
 - D. After 18 months
- ANSWER: B
161. A mother of an infant aged 6 weeks brought her baby to an immunization unit for vaccination. There is a shortage of distilled water at the facility, which is not distributed by the zonal health bureau. What is the most likely vaccine that SHOULD NOT be given based on the above scenario?
- A. Penta vaccine
 - B. Inactivated polio vaccine
 - C. PCV vaccine
 - D. BCG vaccine
- ANSWER: D
162. A district health officer visited a pediatric nurse working in an immunization unit. She has administered several vaccines for infants on this day. The district supervisor assessed whether the given vaccines were delivered in a safe way or not. What is the likely practice performed by the nurse that can be considered unsafe?
- A. Recapping the needle after being used
 - B. Avoid touching the needle before use
 - C. Removing the needle from the vial after being used
 - D. Disposing of used needles in a closed cardboard
- ANSWER: A
163. A pediatric nurse working in an immunization unit received vaccines based on orders from the central vaccine store. In the immunization unit, there is a top-opening refrigerator without baskets. What is the most likely vaccine to be stored at the bottom of the refrigerator?
- A. BCG vaccine
 - B. Penta vaccine
 - C. Oral polio vaccine
 - D. Yellow fever vaccine
- ANSWER: C

164. A 6-week-old female neonate was brought by her mother with complaints of redness and swelling of the injection site since last night. The mother also reported behavioral changes in her baby for an hour. The baby was vaccinated yesterday according to the schedule. The body temperature of 37.7 degrees centigrade is recorded.

What is the most likely vaccine that might cause the above case?

- A. Penta vaccine
- B. Measles vaccine
- C. BCG vaccine
- D. Pneumococcal vaccine

ANSWER: A

165. A supervisor from a district health office visited the immunization unit a day before the facility's vaccination schedule. A nurse arranged the vaccines in the refrigerator, and the supervisor appreciated her effort in keeping the vaccines at the right temperature. What is the most likely temperature range of a refrigerator for the above scenario?

- A. -2degree centigrade to 10 degrees centigrade
- B. -8 degrees centigrade to 2 degrees centigrade
- C. +2 degrees centigrade to +8 degrees centigrade
- D. -10 degrees centigrade to 2 degrees centigrade

ANSWER: C

166. The level of immunization coverage of the polio vaccine in a certain district is 95percent. The district manager appreciated the health extension worker's commitment, contribution, and the cooperation of the community in the district to eradicate the disease.

What is the most likely indicator of the effectiveness of the immunization?

- A. Input indicator
- B. Output indicator
- C. Outcome indicator
- D. Impact indicator

ANSWER: B

167. A pediatric nurse working in a certain kebele catchment planned to reach 90 percent in penta coverage for children under one year old. The target population living in the village is 100. The previous wastage factor was 1, and recommended to add a 10percent buffer stock.

What is the full dose of the above vaccine required for one year?

- A. 297
- B. 375
- C. 760
- D. 800

ANSWER: A

168. In a regional health bureau, the disease prevention and health promotion team wants to utilize reports of all cases of measles in children less than five years of age from a pre-arranged sample of reporting sources. Then, the team coordinated with health facilities to obtain the data.

What is the most appropriate type of surveillance for the above scenario?

- A. IDSR
- B. Active
- C. Passive
- D. Sentinel

ANSWER: D

169. In one of the public high schools, students frequently miss class and spend their time in the surrounding service provider establishment. The school guidance and counseling committee wanted to solve this problem, which can influence the physical and mental health conditions of the students. During observation, the committee noticed coffee houses, shops with hidden rooms, and groceries near the school's compound.

What is the appropriate physical environment that describes this case?

- A. School security
- B. Location of the school
- C. Microclimate of the school
- D. Design and classroom structure

ANSWER: B

170. A mother brought her 1-year-old child to a health center with complaints of diarrhea, vomiting, and loss of appetite. A pediatric nurse diagnosed and treated the case of the child and provided the necessary vaccination to the child.

What is the prioritized level of disease prevention in the above scenario?

- A. Tertiary prevention
- B. Primary prevention
- C. Prevention of exposure
- D. Secondary prevention

ANSWER: D

171. A 24-year-old male cancer patient came to the hospital with complaints of generalized pain and stress. A psychiatrist listened carefully and counseled the patient and provided emotional support to encourage the patient to do gaming, listening to spiritual music, and writing a daily diary.

What is the most likely palliative care in this case?

- A. Radiotherapy
- B. Muscle relaxants
- C. Traditional therapy
- D. Non-pharmacological

ANSWER: D

172. A health extension worker wants to share balanced diet preparation skills for households in a catchment area. Then, the health extension worker selected the common food items used for local cooking for demonstration to the mothers in a group from each household.

What is the most likely criterion used to select the teaching method?

- A. Educational level
- B. Sex of the audience
- C. Teaching objectives
- D. Background of the audience

ANSWER: C

173. A new building under construction was demolished and causing accidents to workers. An emergency team arrived and started activities focusing on saving lives through search and rescue, first aid, medical assistance, and overall disaster assessment.

What phase of emergency management most likely explains the above case?

- A. Recovery
- B. Transition
- C. Emergency
- D. Reconstruction

ANSWER: C

174. A country's public health emergency management committee wants to prepare long-term solutions as a sustained action to minimize the risk to people and property from disasters. Then, the committee designed activities that include hazard identification and mapping, design and construction applications, and structural controls to reduce losses from disaster.
What disaster control measure is most likely implemented in this case?
- A. Mitigation
 - B. Prevention
 - C. Preparedness
 - D. Rehabilitation
- ANSWER: A
175. A team of experts from a health center plans to develop an action plan in order to solve the burden of recurrent outbreaks of diseases. At the beginning, they conducted a detailed situational analysis and came up with evidence suggesting the availability of clear and supportive government policies and the presence of a functional health committee in the communities.
What most likely strategic tool (SWOT) described by the team?
- A. Opportunity
 - B. Strength
 - C. Weakness
 - D. Threat
- ANSWER: A
176. The head of the district health office makes a very small contribution to the planning process. When a decision needs to be made, he usually postpones it for next time and fails to encourage others to do their jobs. Because of such practices, the subordinates in the health office are confused and frustrated by their effective.
What is the most likely leadership style practiced by the district health office head?
- A. Autocratic
 - B. Democratic
 - C. Laissez-Faire
 - D. Republic
- ANSWER: C
177. An Executive director of a hospital delegates planning authority to directors and department heads to formulate their annual plans under the general strategic umbrella of organizational objectives. At the end, a comprehensive planning document was produced by compiling the plans of different service delivery settings of the hospital.
What is the most likely planning approach applied in this case?
- A. Centralized top-down planning
 - B. Bottom-up planning
 - C. Team participatory planning
 - D. Operational planning
- ANSWER: B

178. A leader of an organization arranges a meeting session with the employees by scheduling sufficient uninterrupted time. During a meeting, he genuinely seeks the information, makes eye contact, avoids emotion or attacking others, paraphrases the message, avoids distracting gestures, and asks for clarity questions.

What is the most likely effect of oral communication skills applied by the leader?

- A. Constrictive feedback
- B. Positive feedback
- C. Rapport making.
- D. Active listening

ANSWER: D

179. An enthusiastic pediatric department head aspires to bring change to the quality of care. To be effective in his leadership, the head conducted a preliminary assessment aimed at defining the problem, identifying the problem objective, and diagnosing the cause.

What is the most likely step in decision-making considered by the head?

- A. Investigating the situation
- B. Develop alternatives
- C. Evaluate alternatives
- D. Implement and follow up

ANSWER: A

180. The medical and surgical ward heads conflict of interest in the scholarship opportunity. A director of nursing wants to send the senior ones, but the juniors confronted the director and requested clear guideline that allows a scholarship opportunity for the seniors. After several discussions, the senior nurse agreed to give the opportunity to his juniors to maintain a peaceful working environment.

What is the most likely conflict management style applied in the scenario?

- A. Competition
- B. Avoidance
- C. Compromising
- D. Accommodation

ANSWER: D

181. The Ministry of Education conducted a pilot study that aimed to ensure the effectiveness of student preparation and improve the quality of teaching and learning in all private and public universities of Ethiopia. A quality improvement recommendation document has been developed and distributed to all universities.

What is the most likely type of evaluation applied by the ministry?

- A. Process evaluation
- B. Impact evaluation
- C. Formative evaluation
- D. Summative evaluation

ANSWER: A

182. A district health office head organized an annual staff meeting to evaluate the performance of different sectors. Each sector presented its performed activity report regarding avoiding the wasting of equipment, supplies, ideas, and energy.

What dimension of health care system evaluation is most likely undertaken in this case?

- A. Effectiveness
- B. Efficiency
- C. Adequacy
- D. Competency

ANSWER: B

183. A quality control team coordinator has established a team of experts responsible for identifying services that need prioritization in the organization, identifying available resources, and defining criteria and standards.
What is the most likely quality service audit step adopted by the team?
- A. Measuring the level of performance
 - B. Planning and preparation
 - C. Maintaining improvements
 - D. Making improvements
- ANSWER: B
184. A clinical director has been carrying out frequent facility supervision to follow the implementation of the recommended plan of action and to determine the effectiveness of previous actions taken in developing a range of indicators.
What most likely quality service audit step undertaken in this supervisory practice?
- A. Measuring the level of performance
 - B. Planning and preparation
 - C. Maintaining improvements
 - D. Making improvements
- ANSWER: C
185. An investigator wants to research a topic, "predictors of mortality among TB-HIV co-infected children attending antiretroviral clinic in hospital X". After defending and submitting the proposal, the investigator proceeded with data collection. In due course of data collection investigator revealed that there are no adequate research participants to conduct the study.
What criteria should have been considered before selecting this research title?
- A. Novelty
 - B. Interest
 - C. Feasibility
 - D. Ethics
- ANSWER: C
186. A group of researchers prepared a proposal on: "Practices of early childhood developmental milestone assessment and its associated factors among child health professionals in a referral Hospital".
What is the most likely specific objective for the above research topic?
- A. To identify the practice of child health professionals on child development.
 - B. To understand the practice of child health professionals on child development.
 - C. To identify socio-demographic factors affecting child development.
 - D. To determine the level of practice of child health professionals on child development.
- ANSWER: D
187. A group of researchers wanted to see the effect of parental education on knowledge of infant feeding. The researchers selected a total of 200 parents, took baseline measures, and taught them weekly for six consecutive weeks. Finally, they measured parents' knowledge at the end of the education and compared it with the baseline.
What type of study design did the researchers use in this scenario?
- A. Quasi-experimental
 - B. Randomized controlled trials
 - C. Comparative cross-sectional
 - D. Pre-experimental
- ANSWER: D

188. A group of pediatric and child health nurses is researching unmet needs for family planning among mothers attending antenatal clinics of a health center. While collecting data, one of the study participants refused to give information. However, the researchers told her to participate in the study, otherwise she would not be eligible for the service they are planning to provide. What most likely ethical principle is breached in this case?
- A. Justice
 - B. Autonomy
 - C. Beneficence
 - D. Non-Maleficence
- ANSWER: B
189. A researcher wants to assess the presence or absence of latrines and their status in health facilities, aiming to check and complement the data with reports obtained from the health facilities. The researcher randomly selected 80 health facilities and collected data on the availability and cleanliness of the latrines. What is the most likely data collection method used in this case?
- A. Record review
 - B. Self-administered interview
 - C. Observation
 - D. Face-to-face interview
- ANSWER: C
190. A 15-year-old conscious boy is admitted to a surgical ward with a diagnosis of inguinal hernia. The boy wants to undergo an operation in another hospital. After a while, the nurse in charge presented the informed consent to the mother and explained all pertinent information regarding the procedure. After the discussion, the mother agreed to sign the consent. What is the most likely ethical principle breached in this scenario?
- A. Justice
 - B. Nonmaleficence
 - C. Autonomy
 - D. Beneficence
- ANSWER: C
191. A nurse working in a medical ward discusses with a patient at bed bedside. In concluding the discussion, the nurse stated that 'we always abide by procedure when it brings the greatest possible balance of good over evil. During an emergency, we do not bother about the patient's interest, but making the right decisions. What ethical principle is emphasized in the conclusion?
- A. Justice
 - B. Nonmaleficence
 - C. Autonomy
 - D. Beneficence
- ANSWER: D

192. A two-year-old child came to a pediatric OPD emergency with complaints of abdominal distention, pain, and failure to pass stool for two days. After the assessment, the child was diagnosed with constipation, and an enema was prescribed. A pediatric nurse started a conversation with the mother about the procedure.

What is the next most appropriate role or action of the pediatric nurse?

- A. Check the medical diagnosis
- B. Prescribing an enema
- C. Taking informed consent
- D. Re-check the physical examination

ANSWER: C

193. A nurse was mentoring a 4th-year pediatric nursing student attending their internship on how nursing process steps are applied at the bedside. For the next day, the nurse wanted to see their performance and planned to discuss in detail more based on the progress they made.

What is the most likely role of the nurse in this case?

- A. Supervision
- B. Leadership
- C. Counselling
- D. Teaching

ANSWER: D

194. A four-year-old child patient came to the clinic with nasal bleeding. The child has a frontal headache. His father says that the child fell while playing football. A pediatric nurse stopped the bleeding and cleaned thoroughly. Then counseled the father was counseled to bring back the child if any sign of further complications appears. The father gave thanks to the nurse.

What is the most likely role of this pediatric nurse in this case?

- A. Care giver
- B. Patient advocator
- C. Counsellor
- D. Educator

ANSWER: A

195. A department head observed from the patient's chart that a pediatric nurse conducted wound care using normal saline and hydrogen peroxide. The procedure was performed early in the morning. The patient was crying and defensive while the care was given. Besides, the department head read that the patient had been scheduled for debridement in the afternoon. Which documentation is most INAPPROPRIATE in this scenario?

- A. Wound care was done with NS and H₂O
- B. Wound care is done at 8:00 am
- C. The patient was crying
- D. The patient was grimacing and defensive during wound care

ANSWER: A

196. A pediatric nurse director is discussing with his workmate about proper documentation. They took four different examples from previous recordings and debated on which one showed proper documentation and fulfilled basic rules. Nurse A: wound seems improving; Nurse B: the client is uncooperative; Nurse C: patient respiration is normal; Nurse D: "client said I hate this place". Which one shows the most appropriate and acceptable documentation?
- A. Nurse A
 - B. Nurse B
 - C. Nurse C
 - D. Nurse D
- ANSWER: D
197. A 12-year-old boy came to the clinic with the chief complaint of headache and abdominal cramping. A pediatric nurse is trying to document assessment findings of the child suspected for acute gastroenteritis recorded on the patient's cards. Which choice most likely showed proper documentation of the finding of the child's case?
- A. Normal skin turgor
 - B. Vomited two times since 6 hours
 - C. Gave two cups of drinking water
 - D. Raised pulse rate and temperature
- ANSWER: B
198. A 12-year-old diabetic child, the child's mother, and a nurse were communicating on the child's meal plan. The nurse was using clear but very strong words. The mother was shy while the nurse kept eye contact. The mother asked for the use of an orange instead of a mango, explaining the child's previous preference. The nurse told the mother to use a mango, not an orange. The child is attending a conversation between the nurse and his mother. What effective communication quality is most likely lost in the conversation?
- A. Using clear words
 - B. Mother's eye contact
 - C. Using strong words
 - D. Ignoring client choice
- ANSWER: D
199. A mother came to hospital with her 7-year-old child who has a problem of a closed vaginal orifice congenitally. A pediatric male nurse was assessing for any problems the child faced, but the mother kept silent about telling him. The male nurse was confused about what was going on. So the nurse called another female nurse to communicate with the mother. The mother told the female nurse that the male nurse was talking about the health condition of his previous patient, specifying the name of the patient. The mother added that it is because of that she got afraid to tell him the case of her daughter. What quality of effective communication is the first nurse lacking?
- A. Rigidity
 - B. Flexibility
 - C. Truthfulness
 - D. Confidentiality
- ANSWER: D

200. A pediatric nurse was caring for a 2-year-old child with intermittent diarrhea for two days. The nurse was asking the child's father about the condition. He was listening to the father's explanations attentively, noticing hand and facial expressions of the father. The nurse was repeatedly asking the father to repeat some unfamiliar words. The father was a native speaker of the Somali language, but can speak and understand the language that the nurse is speaking.

What is the most likely barrier to communication between the nurse and the father?

- A. Language
- B. Background
- C. Listening pattern
- D. Gesture taking

ANSWER: B



ETHIOPIAN HEALTH PROFESSIONALS LICENSING EXAMINATION(EHPLE) PRACTICE TEST BOOKLET 2025

PEDIATRIC & CHILD HEALTH NURSING



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